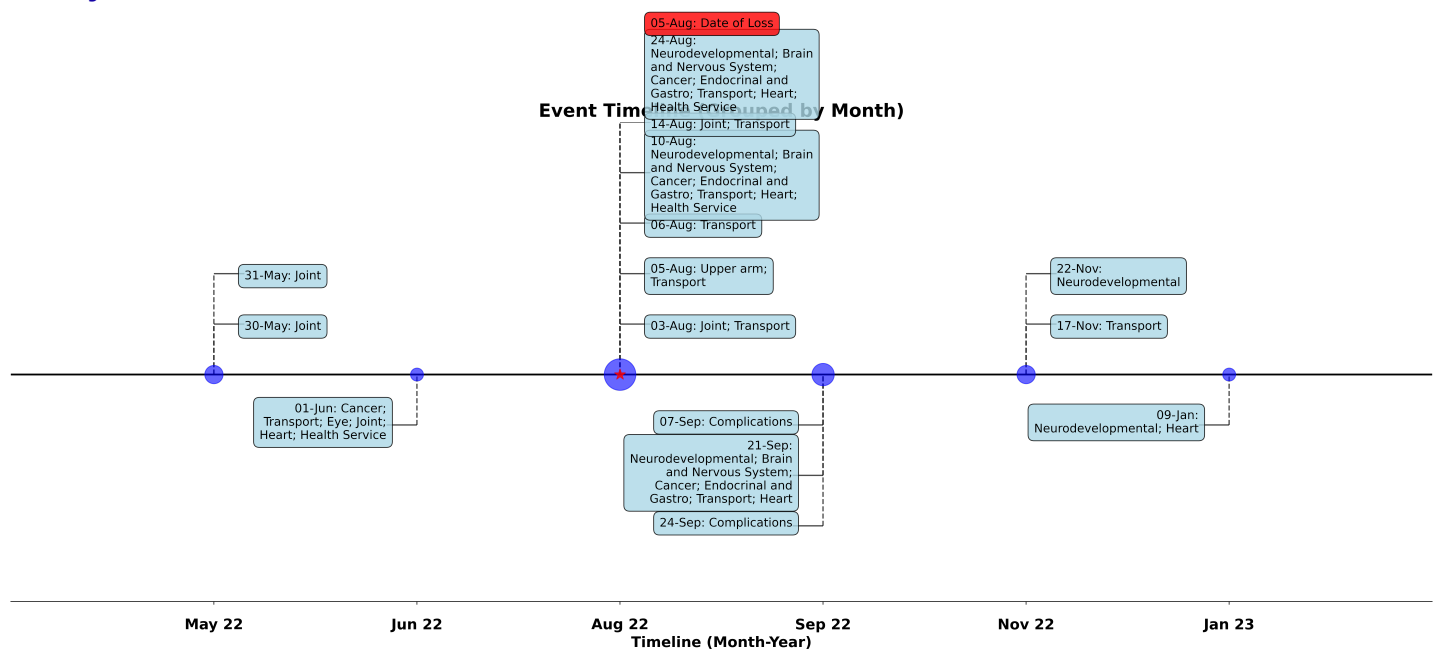


Medical Summary

Claim File Name: GP-File-2
Total Pages: 69
Date of Loss or Date of Incident: 08/05/2022

1. Common

1.1 Major Events Timeline



1.2 List of Medical Conditions

1.2.1 Chronological Visits

Note: Medical conditions are categorized as either a single occurrence in (Grey) or multiple occurrences (Black) for accurate tracking and reporting.

Before incident

Date	Abstract
05/30/2022	Patient is receiving physical therapy for pain in left shoulder, experiencing ongoing discomfort rated 4-5/10. Treatment focuses on neuromuscular reeducation to improve alignment, scapular stability, and joint mechanics, with the patient reporting gradual improvement. Pain In Left Shoulder[1] Providers Involved: Maria Fe Godbey, Physical Therapy of Phenix City
05/31/2022	Patient is receiving physical therapy for left shoulder pain, with pain intensity now reduced to 3/10. The treatment plan includes neuromuscular reeducation and therapeutic exercises specifically targeting improved alignment, enhanced scapular stability, and better joint mechanics. Pain In Left Shoulder[1] Providers Involved: Maria Fe Godbey, Physical Therapy of Phenix City
06/01/2022	Patient is a male experiencing acute left shoulder pain following a motor vehicular accident where his vehicle was "T-boned" on his side. He was treated in the emergency room and underwent X-ray

	examination. Prior to the accident, he had no shoulder issues, but has a medical history including high blood pressure, stroke affecting his right upper extremity, and a childhood brain tumor. Patient primarily relies on his left upper extremity and was referred to Physical Therapy of Phenix City for treatment and evaluation. He currently takes Lisinopril and Metformin, exercises three days weekly, and aims to improve his condition through regular physical therapy exercises. Pain[1] , Motor Vehicular Accident[1] , Accident[1] , Stroke[1] [2] , Brain Tumor[1] , Blind[1] , High Blood Pressure[1] Providers Involved: Maria Fe Godbey, Physical Therapy of Phenix City Patient Involved: Roger Pouncey
08/03/2022	Patient experienced pain in the left shoulder following a motor vehicle accident. During the visit with Dr. Dennis K Harden at Piedmont Physicians, the patient received assessment and treatment for the shoulder pain resulting from the traffic accident injury. Pain In Left Shoulder[1] , Person Injured In Unspecified Motor-Vehicle Accident, Traffic, Initial Encounter[1] Providers Involved: MD, Dennis K Harden

After incident

Date	Abstract
08/05/2022	Patient was a 36-year-old male who presented to the ER via ambulatory transport with a left shoulder injury from a motor vehicle collision where he was the driver. He experienced acute pain, decreased range of motion, and difficulty raising his arm, with symptoms emerging approximately one hour prior to arrival. X-ray examination of the left clavicle revealed no acute fractures or dislocations, with intact glenohumeral and acromioclavicular joints. He was treated for shoulder pain in the emergency department and advised to follow up with a private physician as needed and to monitor for potential worsening of his condition. Dt Mva[1] , Motor Vehicle Collison[1] , Driver Injured In Collision With Unspecified Motor Vehicles In Traffic Accident, Initial Encounter[1] , Motor Vehicle Collision Injury[1] , Left Clavicle Injury[1] , Driver Injured In Collision[1] Providers Involved: Cameron, OSHINOWO ABAYOMI, Kersey, Jack Hughston Memorial Hospital, ABAYOMI, OSHINOWO Patient Involved: Jack Hughston
08/06/2022	Patient was involved in a motor vehicle collision as a driver, resulting in left shoulder pain. They received medical screening and were treated with ketorolac, methocarbamol, and dexamethasone for their injuries. The patient was discharged in good condition with follow-up instructions to continue care at home. Driver Injured In Collision With Unspecified Motor Vehicles In Ao Traffic Accident, Initial Encounter[1]
08/10/2022	Patient was involved in a motor vehicle accident where another vehicle crossed the centerline and struck his car. He was transported by ambulance to Memorial, underwent x-rays showing no acute fractures, and is now experiencing left shoulder pain during follow-up. He has an extensive medical history including brain tumor requiring multiple surgeries, hypertension, major depressive disorder, obesity, obstructive sleep apnea, and testicular hypofunction. His surgical history also includes an appendectomy and wisdom tooth extraction. For the current shoulder pain, he has been prescribed tiZANidine 4mg tablets, three times daily for 10 days. Motor Vehicle Accident[1] , Brain Tumor[1] , Hc[1] , Hypertension[1] , Mdd[1] , Major Depressive Disorder[1] , Obesity[1] , Obstructive Sleep Apnea[1] , Cerebrospinal Fluid Drainage Device[1] , Testicular Hypofunction[1] , Motor Vehicle Accident Injuring Restrained Driver, Initial Encounter[1] Providers Involved: ChartPro Incorporated, Piedmont Physicians at Stadium Drive, MD, Dennis K Harden Patient Involved: Dennis K Harden

08/14/2022	Patient presented with left shoulder pain resulting from a motor vehicle accident. During the visit with Dr. Dennis K Harden at Piedmont Physicians Columbus Stadium Drive Primary Care, the patient received care for the active shoulder condition. This was a follow-up appointment addressing ongoing treatment for injuries sustained in the traffic accident. Pain In Left Shoulder[1] , Person Injured In Unspecified Motor-Vehicle Accident, Traffic, Subsequent Encounter[1] Providers Involved: MEDMONT PHYSICIANS, PIEDMONT PHYSICIANS COLUMBUS STADIUM DRIVE PRIMARY CARE, Dennis K Harden
08/24/2022	Patient was involved in a motor vehicle accident approximately 3 weeks ago, resulting in left shoulder pain. After initially presenting to the emergency room with no fracture, he later visited the office with severe shoulder pain, decreased range of motion, and sleep difficulties. The patient has a complex medical history including a brain tumor requiring multiple surgeries, a cerebrospinal fluid drainage device, hypertension, major depressive disorder, obesity, obstructive sleep apnea, and testicular hypofunction. His surgical history also includes an appendectomy and wisdom tooth extraction. Following this motor vehicle accident, the patient was referred for routine physical therapy evaluation and treatment, with a follow-up scheduled after approximately 4 weeks of therapy. Motor Vehicle Accident[1] , Brain Tumor[1] , Hypertension[1] , Mdd[1] , Major Depressive Disorder[1] , Obesity[1] , Obstructive Sleep Apnea[1] , Cerebrospinal Fluid Drainage Device[1] , Testicular Hypofunction[1] , Motor Vehicle Accident Injuring Restrained Driver, Initial Encounter[1] Providers Involved: ChartPro Incorporated, Piedmont Physicians at Stadium Drive, Dennis K. Harden, MD, Dennis K Harden Patient Involved: Dennis K Harden
09/07/2022	Patient will undergo physical therapy to improve scapular stability through therapeutic exercises and Electrical Stimulation. The comprehensive treatment plan spans 8 visits over 4 weeks, with a focus on neuromuscular reeducation. The ultimate goal is for Patient to master the home exercise program independently with proper technique. Electrical Stimulation[1] Providers Involved: Maria Fe Godbey, Dennis Harden
09/21/2022	Patient was involved in a motor vehicle accident approximately 3 weeks ago, resulting in left shoulder pain. After initially visiting the emergency room where no fracture was detected, he presented with severe shoulder pain, decreased range of motion, and sleep difficulties. The patient has a complex medical history including a brain tumor requiring multiple surgeries, hypertension, major depressive disorder, obesity, obstructive sleep apnea, and testicular hypofunction. He has also previously undergone an appendectomy and wisdom tooth extraction. For the current shoulder injury, an MRI without contrast was ordered, with plans for physical therapy and potential follow-up imaging. Motor Vehicle Accident[1] , Brain Tumor[1] , Hypertension[1] , Major Depressive Disorder[1] , Obesity[1] , Obstructive Sleep Apnea Of Fluid[1] , Testicular Hypofunction[1] , Motor Vehicle Accident Injuring Restrained Driver, Subsequent Encounter[1] Providers Involved: ChartPro Incorporated, Donnie K. Hardon, MD, Piedmont Physicians, Piedmont Physicians at Stadium, Piedmont MEALHCARE, Dennis K Harden
09/24/2022	Patient underwent an MRI of the upper extremity joint without contrast. The diagnostic examination was performed at Bridgeway Diagnostic Radiology with a total charge of \$1,545.00. Mri Joint Upr Extrem W/O Dye[1] Providers Involved: ROHENAI, LLC, BRIDGEWAY DIAGNOSTIC RADIOLOGY
11/17/2022	Patient was involved in a motor vehicle collision as a driver and sustained left shoulder pain. After medical screening, the patient was discharged home in good condition with pain medications and instructions to follow up with a private physician if symptoms worsen.

	Driver Injured In Collision With Unspecified Motor Vehicles In Ao Traffic Accident[1] , Motor Vehicle Collision Injury[1]
11/22/2022	Patient presented with recurrent moderate major depressive disorder alongside several comorbidities including brain tumor, hypertension, obesity, obstructive sleep apnea, and testicular hypofunction. The patient's medical history is significant for multiple surgical interventions, most notably brain surgery. Social history indicates he is currently single. Management focuses on addressing both psychiatric and multiple medical conditions concurrently. Mdd[1] Providers Involved: ChartPro Incorporated, MD, Donnie K. Hardon, Piedmont MEALTHCARE
01/09/2023	Patient, who is Claustrophobic, is preparing for an MRI scan and has been instructed to remove all metal objects including bobby pins, medication patches, hearing aids, and dental work containing metal. The patient has a history of Hypertension and CVA (Cerebrovascular Accident), which requires special medical considerations during the imaging procedure, particularly regarding the potential use of contrast agents. Claustrophobic[1] , Hypertensive[1] , Cva[1] Providers Involved: Piedrens Patient Involved: Mary Mills

1.3 ICD Groups

[illegible]

	I69 - Stroke I60 - Hc	Pre-existing Condition Pre-existing Condition
F01-F99 Neurodevelopmental	F33 - Mdd F33.1 - Major Depressive Disorder F32 - Mdd F40 - Claustrophobic	Pre-existing Condition Pre-existing Condition Pre-existing Condition Pre-existing Condition
E00-E89 Endocrinal and Gastro	E29 - Testicular Hypofunction E66 - Obesity	Pre-existing Condition Pre-existing Condition
G00-G99 Brain and Nervous System	G47.33 - Obstructive Sleep Apnea G47 - Obstructive Sleep Apnea Of Fluid	Pre-existing Condition Pre-existing Condition
Z00-Z99 Health Service	Z04 - Accident Z98.2 - Cerebrospinal Fluid Drainage Device	Accident Pre-existing Condition
T66-T88 Complications	T85 - Electrical Stimulation T84 - Mrl Joint Upr Extrem W/O Dye	Accident Pre-existing Condition
H00-H59 Eye	H54 - Blind	Pre-existing Condition

1.4 Medical Necessity

Condition: Brain Tumor

Treatment Guidance (Mayo Clinic)

- *First-line Treatments:* Surgery, radiation therapy, chemotherapy
- *Second-line / Alternative Treatments:* Targeted drug therapy, immunotherapy
- *Supportive / Lifestyle Therapies:* Rehabilitation, supportive care
- *When to Consult a Specialist:* Upon diagnosis or recurrence
- *Associated Risks or Complications:* Neurological deficits, increased intracranial pressure

Treatment Plan Validation Against Guidance:

- *Treatments Documented in Summary:* Brain surgery (performed 4 times)
- *Missing Treatments (per guidelines):* Radiation therapy, chemotherapy (not mentioned)
- *Outdated or Risky Interventions:* None
- *Deviation from Guidelines:* No deviation

Condition: Hypertension

Treatment Guidance (Mayo Clinic)

- *First-line Treatments:* Lifestyle changes, thiazide diuretics, ACE inhibitors
- *Second-line / Alternative Treatments:* Calcium channel blockers, beta blockers
- *Supportive / Lifestyle Therapies:* Diet modification, regular exercise
- *When to Consult a Specialist:* Resistant hypertension or complications
- *Associated Risks or Complications:* Stroke, heart disease

Treatment Plan Validation Against Guidance:

- *Treatments Documented in Summary:* Lisinopril (ACE inhibitor)
- *Missing Treatments (per guidelines):* Lifestyle changes (partially mentioned)
- *Outdated or Risky Interventions:* None
- *Deviation from Guidelines:* No deviation

Condition: Major Depressive Disorder

Treatment Guidance (Mayo Clinic)

- *First-line Treatments:* SSRIs, psychotherapy
- *Second-line / Alternative Treatments:* SNRIs, atypical antidepressants
- *Supportive / Lifestyle Therapies:* Exercise, support groups

- *When to Consult a Specialist:* Severe symptoms or suicidal ideation
- *Associated Risks or Complications:* Suicide, self-harm

Treatment Plan Validation Against Guidance:

- *Treatments Documented in Summary:* None
- *Missing Treatments (per guidelines):* SSRIs, psychotherapy
- *Outdated or Risky Interventions:* None
- *Deviation from Guidelines:* Deviation exists

Condition: Obesity

Treatment Guidance (Mayo Clinic)

- *First-line Treatments:* Diet and exercise, behavioral therapy
- *Second-line / Alternative Treatments:* Medications (e.g., orlistat), surgery
- *Supportive / Lifestyle Therapies:* Support groups, counseling
- *When to Consult a Specialist:* BMI > 40 or complications
- *Associated Risks or Complications:* Diabetes, cardiovascular disease

Treatment Plan Validation Against Guidance:

- *Treatments Documented in Summary:* None
- *Missing Treatments (per guidelines):* Diet and exercise, behavioral therapy
- *Outdated or Risky Interventions:* None
- *Deviation from Guidelines:* Deviation exists

Condition: Obstructive Sleep Apnea

Treatment Guidance (Mayo Clinic)

- *First-line Treatments:* CPAP, lifestyle changes
- *Second-line / Alternative Treatments:* Oral appliances, surgery
- *Supportive / Lifestyle Therapies:* Weight loss, positional therapy
- *When to Consult a Specialist:* Diagnosis or treatment failure
- *Associated Risks or Complications:* Cardiovascular problems

Treatment Plan Validation Against Guidance:

- *Treatments Documented in Summary:* None
- *Missing Treatments (per guidelines):* CPAP, lifestyle changes
- *Outdated or Risky Interventions:* None
- *Deviation from Guidelines:* Deviation exists

Condition: Testicular Hypofunction

Treatment Guidance (Mayo Clinic)

- *First-line Treatments:* Testosterone replacement therapy
- *Second-line / Alternative Treatments:* Clomiphene citrate
- *Supportive / Lifestyle Therapies:* Regular monitoring
- *When to Consult a Specialist:* Diagnosis or treatment failure
- *Associated Risks or Complications:* Infertility, osteoporosis

Treatment Plan Validation Against Guidance:

- *Treatments Documented in Summary:* None
- *Missing Treatments (per guidelines):* Testosterone replacement therapy
- *Outdated or Risky Interventions:* None
- *Deviation from Guidelines:* Deviation exists

1.5 Medication List

Medication Name	Dosage	Frequency	Route
Dexamethasone	4 mg	-	IM
ketorolac	60 mg	-	IM
Methocarbamol	1 grams	pd	PO
Ibuprofen	1 Tablet	every 8 hours As needed	ORAL
Prednisone	1 Tablet	once daily	ORAL
Robaxin	1 Tablet	every 12 hours As needed	ORAL
amLODIPine	1 tablet	daily	by mouth
NORVASC	1 tablet	daily	by mouth
cholecalciferol, vitamin D3	1,000 Units	daily	by mouth
lisinopril-hydrochlorothiazide	1 tablet	daily	by mouth
ZESTORETIC	1 tablet	daily	by mouth
metFORMIN	500 mg	2 (two) times daily with meals	by mouth
GLUCOPHAGE	500 mg	2 (two) times daily with meals	by mouth
naproxen	1 tablet	2 (two) times daily with meals	by mouth
NAPROSYN	1 tablet	2 (two) times daily with meals	by mouth
phentermine	1 tablet	every morning before breakfast	by mouth
ADIPEX-P	1 tablet	every morning before breakfast	by mouth
tiZANidine	1 tablet	in the morning	by mouth
ZANAFLEX	1 tablet	in the morning	by mouth

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